Bevacizumab + capecitabine

NCCP 00623 · Draft for clinical, pharmacy and patient/PPI review

PRACTICAL TREATMENT GUIDE

Tablets, visits and practical planning

A simple guide to what happens before treatment, on treatment days and at home. Your own service confirms appointments and prescriptions.

STEP 1

Before the first dose

Confirm DPD results, blood tests, kidney/liver checks and the treatment aim. Keep a current medicine list.

STEP 2

Get the tablet plan

Check the number of 150 mg and/or 500 mg tablets in each dose and exactly which days to take them. Bring questions to the oncology pharmacist.

STEP 3

Plan your visits

The first bevacizumab infusion usually runs over 90 minutes; later infusions may shorten if tolerated. Checks and preparation add time. A cannula or existing suitable line is used; there is no take-home pump in this regimen.

STEP 4

Take tablets with food

Take the prescribed morning and evening doses with or within 30 minutes of food, swallowed whole with water. If swallowing is difficult, contact pharmacy for a safe plan.

STEP 5

Know when to pause

Significant diarrhoea, painful mouth or hands/feet, fever, sudden illness or dehydration: stop tablets and call. A missed or vomited dose is not replaced.

STEP 6

Review and next steps

Do not restart after a toxicity break without instructions. Bring your symptom record and remaining tablets if requested. Return unused tablets to pharmacy.

▼ Keeping tablets safe

Keep the original labelled packaging away from children. Follow the storage label and pharmacy handling instructions. Do not share tablets or discard them in household waste.

▼ If treatment is interrupted

Tell the team which doses were missed and why. Never extend the course, double a dose or move tablet days yourself. Ask for a revised written plan.

▼ Questions about your appointments

Ask about transport, blood-test location, pharmacy collection and a named contact for appointment changes. Booking arrangements differ between centres.

▼ Venous access and procedures

Bevacizumab may be given through a cannula or a suitable existing central line. A port is not automatically required. Discuss any planned line insertion, dental work or surgery with oncology because bevacizumab affects wound healing; the team sets any treatment pause.

Sources: NCCP 00623 v3a: official regimen · eviQ: taking capecitabine and managing symptoms (NCCP defines the Irish schedule). Draft for clinical, pharmacy and patient/PPI review.